

URGENT ENT / ER EVALUATION SUMMARY

Concern for Left Acquired Cholesteatoma — Middle Ear / Mastoid Pathology

Patient: Daniel S. May **DOB:** 10/11/1989 (Age 36) **MRN:** 8026121 (MEEI Otolaryngology)
Prepared: July 14, 2026 **Provider:** Self-prepared summary for ER/ENT staff (full records & imaging available)

RELEVANT CLINICAL HISTORY & SYMPTOMS

Long-standing left ear / sinonasal disease: History of nasal fractures with two septoplasties and multiple sinus surgeries. Frequent sinus infections (~6/year) for >10 years. Chronic left Eustachian tube dysfunction (ETD) with ear fullness, popping/crackling, tinnitus, and conductive hearing issues predating current job (started early 2026).

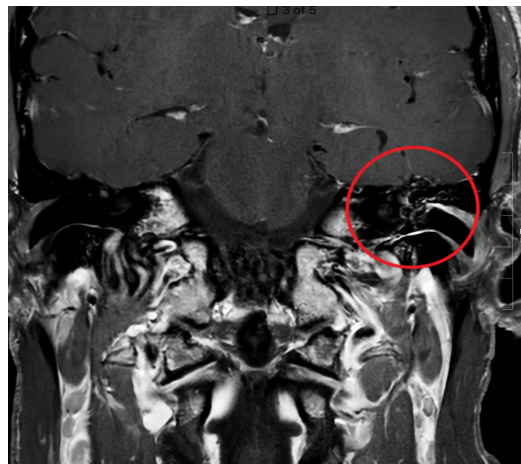
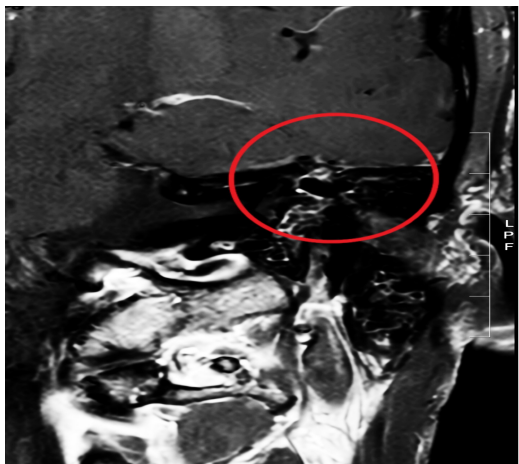
- **~August 2025:** CT Head/Face (baseline): mild mucosal thickening (right maxillary prominent); used for comparison showing later sinonasal improvement but new left middle ear/mastoid changes.
- **~Oct 2025:** Left tympanostomy (PE) tube placed for documented ETD (negative pressure on audiogram) and persistent aural symptoms affecting sleep/function.
- **Nov 2025 – Jan 2026:** Persistent left ear fullness, yellowish-green drainage despite tube, variable right ear symptoms. Budesonide nasal rinses helpful for nose but limited ear relief. Diagnosed with rhinitis medicamentosa (discontinued oxymetazoline).
- **Jan 23, 2026:** Left acute otitis media (tube in place, opacified TM, middle ear fluid). Treated with Augmentin + ofloxacin drops.
- **Feb–Mar 2026:** New onset severe dizziness/vertigo (spinning + off-balance), nausea, lightheadedness, brain fog, intermittent bilateral tinnitus (worse left), popping, transient hearing fluctuations. Symptoms severely impacted work and daily life. One prior vertigo episode in 20s after flying/swimming.
- **Current (July 2026):** Slow but steadily increasing severity of symptoms over the past year. Now: extreme left ear fullness (non-functioning PE tube), ongoing dizziness/nausea (months), frequent tinnitus/crackling, plus significant cognitive decline/brain fog, tingling/numbness, and vision flashes — raising concern for possible CNS/brain infection (MRI left hemipons lesion + chronic mastoid disease; ID referral in process). Notably over past 4 weeks: minimal ability to function normally, several missed work days, and official request today for personal/medical leave from employer. Using budesonide rinses + Bebird monitoring.

KEY IMAGING FINDINGS (Patient-Annotated Images Below)

April 22, 2026 — MRI Brain (IAC Protocol)

Indication: Recurrent brief vertigo episodes triggered by nose blowing or sinus irrigation; evaluate for structural IAC/CPA pathology.

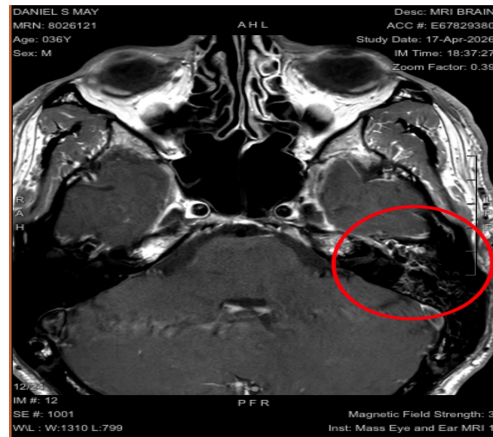
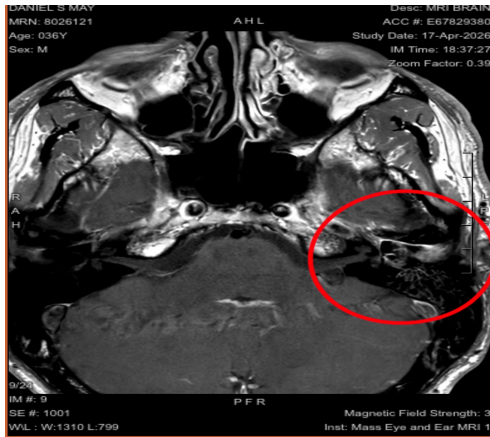
Relevant findings: No CPA or IAC mass. Small volume **left mastoid fluid**. Tiny focus of susceptibility in left hemipons with faint enhancement (reported as low-flow vascular malformation). Few nonspecific periventricular/subcortical white matter FLAIR hyperintensities. Scattered mild paranasal sinus mucosal thickening. **Red circles on images highlight left middle ear / mastoid / epitympanic region of interest.**



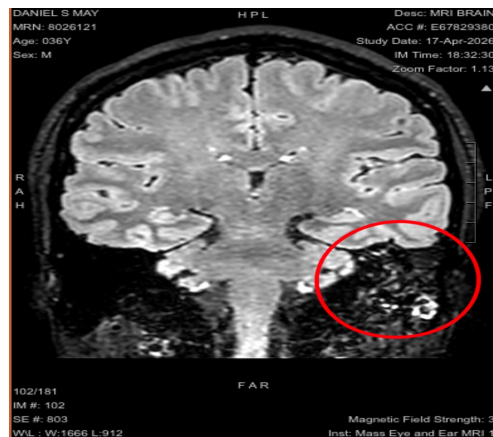
MRI (April 17, 2026, red circles): Left middle ear / epitympanic / mastoid region of interest.

Additional Official MRI Screenshots — April 17, 2026 (MEEI report)

These official screenshots from the April 17, 2026 MRI Brain (IAC protocol) further illustrate the left mastoid fluid and parenchymal findings. Presented as two sets.



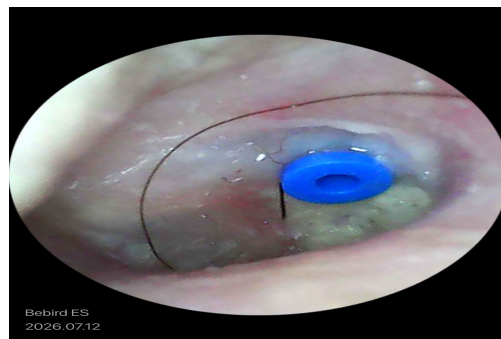
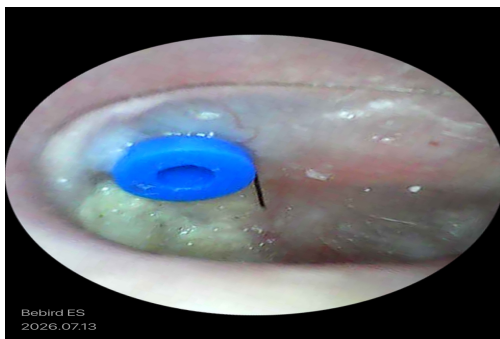
Set 1 — Axial views (IAC / petrous level): Demonstrates left mastoid air cell opacification/fluid and anatomy of the internal auditory canals.



Set 2 — Coronal brain views: Show periventricular and subcortical white matter signal changes consistent with reported nonspecific FLAIR hyperintensities.

July 10–13, 2026 — Personal Otoscope / Ear Camera (Home Visualization)

Patient self-performed otoscopy showing **left PE tube in tympanic membrane** (blue flange visible). Surrounding and behind the drum: areas of white/grayish tissue and possible mass-like appearance noted by patient, raising concern for cholesteatoma. Images dated July 12 & 13, 2026. Additional photos/videos available from patient.



Personal otoscopy (July 12 & 13, 2026): Blue PE tube visible in left TM. White/grayish tissue around and behind the drum (patient concern for cholesteatoma).

Left Ear (Exterior) - Abnormal Appearance — June 6, 2026

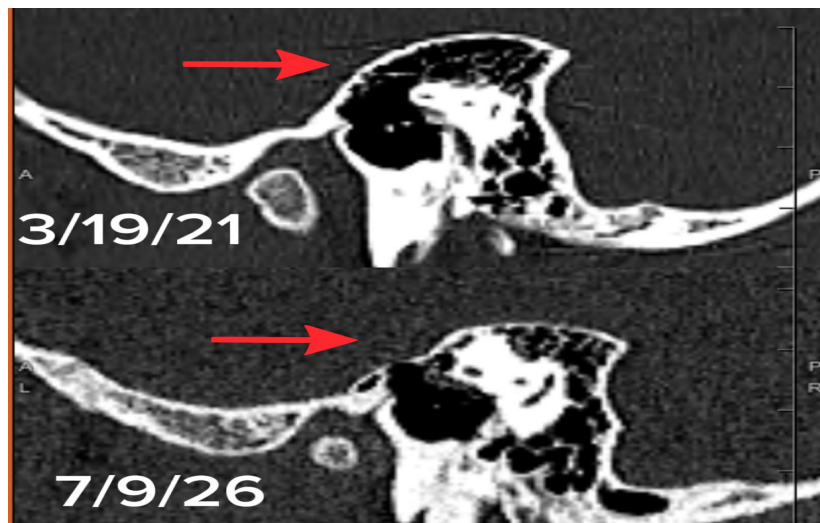
Patient photo showing abnormal skin appearance behind and below the left ear (redness, discoloration, and irritated patches). This exterior finding was noted around the time of ongoing ear symptoms.



Left ear exterior (June 6, 2026): Redness and abnormal skin changes behind and inferior to the left ear, noted during period of active ear symptoms.

July 9, 2026 — CT Head/Face (Surgical Planning Protocol) — Comparison to Prior

Compared to prior CT (Aug 2025 and earlier 2021 imaging). **New findings:** Mild opacification of left mastoid air cells; **new soft tissue opacification in left epitympanic fossa**; thickening of left tympanic membrane. Radiologist impression: may represent sequela of prior otitis media. Sinonasal mucosal thickening improved vs prior. **Red arrows on images point to the left epitympanic / attic region showing interval change / new soft tissue.**



CT (7/9/26 vs prior): Red arrows — new left epitympanic soft tissue opacification + TM thickening (interval change).

CLINICAL IMPRESSION & CONCERN FOR CHOLESTEATOMA

High clinical suspicion for left acquired cholesteatoma based on:

- Chronic/recurrent left middle ear suppuration and ETD despite PE tube placement.
- Non-functioning tube with persistent fullness, drainage, and otoscopic evidence of abnormal tissue/mass behind TM.
- **New soft tissue in left epitympanic fossa** and TM thickening on recent CT (interval change from prior imaging, including Aug 2025 baseline).
- Left mastoid fluid on MRI and clinical vestibular symptoms (dizziness, vertigo, nausea) that may relate to labyrinthine involvement or pressure effects.
- Long history of sinonasal disease with eustachian tube compromise providing pathway for retraction pocket / cholesteatoma formation.
- **Recent significant cognitive decline/brain fog worsening, tingling/numbness, and vision flashes** — raising concern for possible CNS/brain infection involvement (context of MRI left hemipons lesion): ID referral underway.

■ **URGENT REQUEST:** Prompt ENT consultation, otomicroscopy / binocular examination, possible repeat audiometry, culture of any drainage, and consideration for urgent surgical intervention (tympanomastoidectomy, cholesteatoma excision, possible ossicular reconstruction). Patient is prepared with all prior records, MRI, CT, and Bebird media.

APPENDIX: Comprehensive Medical History Summary

Testing & Imaging

Aug 11, 2025 — CT Face (ED, Panara)

- Mild mucosal thickening throughout paranasal sinuses, most prominent right maxillary with aerated secretions.
- Comparison to 2021 CT showed similar findings.

Oct 4, 2025 — Audiology (MEEI)

- Left Eustachian tube dysfunction with negative middle ear pressure.
- Normal hearing thresholds; absent ipsilateral acoustic reflexes right ear.
- Speech testing consistent with conductive overlay left ear.

Mar 27, 2026 — Vestibular VOG (Bery, MEEI Vestibular Lab)

- No spontaneous nystagmus with fixation; normal pursuit/OKN.
- Mild left-beating nystagmus on head-shaking and left Dix-Hallpike (minimal symptoms).
- Very mild right-beating on right Dix-Hallpike.

Apr 17, 2026 — MRI Brain IAC with/without contrast (Bery)

- No CPA or IAC mass.
- Tiny focus of susceptibility in left hemispons with faint enhancement (consistent with low-flow vascular malformation).
- Few nonspecific periventricular/subcortical white matter FLAIR hyperintensities.
- Small volume left mastoid fluid.

Jun 30, 2026 — Sinus Culture/Gram Stain

- Negative (normal flora). No growth.

Jul 1, 2026 — Allergy/Immunology Testing (Brennan, BWH)

- Negative IgE for White Oak, Timothy Grass, Short Ragweed, Penicillium notatum, Mugwort, Mouse Epithelium (all <0.10 kU/L).

Jul 10, 2026 — CT Face/Sinus without contrast (Lee)

- Improved sinonasal mucosal thickening vs Aug 2025 (frontal/ethmoid/maxillary clear or improved).
- New/mild opacification left mastoid air cells.
- New soft tissue opacification left epitympanic fossa.
- Thickening left tympanic membrane (possible sequela prior OM).
- Leftward nasal septal deviation.

Health Visit Clinical Notes

July 16, 2025 — ED Otolaryngology (Dahlberg, MEEI)

- Chief complaint: Sinus pain; history of >15 years sinus trouble, multiple sinus surgeries >10 yrs ago, >7 months bilateral sinus congestion L>R with intermittent ear fullness.
- Using nasal rinse with hydrogen peroxide (Jan-May 2025) which “cleaned a lot of sticky stuff out”; recently trying in ears causing popping but no improvement.
- Assessment: Chronic sinusitis of both maxillary sinuses + Eustachian tube dysfunction.
- Plan: Start nasal steroids + short course oxymetazoline (with dependence caution); refer to Rhinology clinic.

August 15, 2025 — Rhinology Office Visit (Gray, MEEI)

- 35 y.o. commercial real estate broker with chronic rhinosinusitis; ED visits Jul/Aug 2025 for congestion.
- Aug 2025 Sinus CT: mild mucosal thickening, most prominent right maxillary with secretions.
- History: frequent sinus infections (~6/yr) & ear blockage >10 yrs; 2 prior septoplasties for nasal fractures.
- Started Navage + hydrogen peroxide + frequent topical decongestants with improvement clearing thick/sticky (yellow-green or bloody) mucus.

September 17, 2025 — Rhinology Follow-up (Gray, MEEI)

- Follow-up for chronic rhinosinusitis; similar history and ongoing thick mucus production despite Navage + H2O2 and decongestants.

October 3, 2025 — Otolaryngology (Krause, MEEI)

- Referred by Dr. Gray for bilateral ear fullness and Eustachian tube dysfunction (L > R), popping/crackling.
- History of chronic sinus issues and chronic ear infections every 1-2 months, worsening past year.
- Using budesonide nasal rinses (helping sinuses but not ears); sensation of sticky/sappy substance in Eustachian tubes.

October 31, 2025 — Otolaryngology (Krause, MEEI)

- Persistent left ear fullness (“stuck” and “sticky”), chronic ear infections worsening; hearing test showed pressure in left ear.
- Using Mucinex, nasal sprays, budesonide; nasal rinses helping clear thick/sticky mucus (sometimes bloody).

November 21, 2025 — Otolaryngology (Krause, MEEI)

- Left PE tube placed ~3 weeks prior; initial mild improvement (“breathe a little more”) then persistent fullness and yellowish-green drainage.
- Using Q-tip for small morning discharge; budesonide helping nasal but not aural symptoms.

December 5, 2025 — Otolaryngology (Krause, MEEI)

- Persistent left ear fullness after tube (“tight rubber bands” / “strong sticky web”); fluctuating hearing volume.
- Completed Augmentin + ear drops; 10-20% improvement, no current drainage; history of oral prednisone helping sciatica previously.

January 23, 2026 — Otolaryngology (Krause, MEEI)

- Left acute otitis media with tube in place; persistent drainage, fullness, mild pain, tinnitus (L > R).
- Treated with Augmentin + ofloxacin drops; difficulty scheduling ID appointment.

March 20, 2026 — Otolaryngology (Krause, MEEI)

- New significant dizziness/vertigo (spinning + off-balance), nausea, lightheadedness, flu-like symptoms ~3 weeks prior (post-snowstorm); severely impacting work and daily life.
- Gradual improvement in nasal obstruction and left aural fullness with budesonide rinses; intermittent bilateral tinnitus (L worse), popping, transient hearing volume decreases.

May 18, 2026 — Infectious Disease Follow-up (Marglin)

- Follow up from previous appointment.
- Recommendation to get 2nd opinion ENT evaluation.

March 27, 2026 — Vestibular / Oto-neurology (Bery, MEEI)

- Dizziness triggered by nose blowing or sinus irrigation; prior two migraines summer 2025.
- Longstanding ear and sinus issues; VOG showed mild head-shaking and left Dix-Hallpike nystagmus.

March 26, 2026 — Infectious Disease (Marglin)

- Evaluation for continued symptoms potentially pertaining to autoimmune disease.
- Prescribed strong antibiotic to take 1x per week for 12 weeks.

June 29, 2026 — ED Otolaryngology (Rahmati, MEEI)

- Persistent bilateral ear fullness (L worse) with tube in place (dry, no infection); bilateral otalgia; multiple oral ulcers (tongue, hard palate, cheek) since ~June 26.
- Chronic sinus congestion >1 year; thick mucus; irregular stools, nausea/vomiting; concern for possible immune deficiency/autoimmune.
- Nasal endoscopy: left septal deviation, hypertrophic inferior turbinates, clear middle meatus/sphenoethmoidal recesses bilaterally.
- Plan: Refer to otologist, allergy/immunology for immune workup, GI; steroid paste for oral ulcers.

June 30, 2026 — Otolaryngology (Lee, BWH)

- 2+ years persistent nasal fullness, thick sticky mucus (large cohesive plugs, 100-200 nose blows/day), ear and throat involvement; worsened recently, left side prominent.
- Self ear camera exam showed yellowish-white mass and drainage; ear tube placed but no relief.

July 1, 2026 — Allergy & Immunology (Brennan, BWH)

- Referred for chronic sinusitis without polyps, thick elastic yellow/green mucus, concerns for allergy, biofilm, immunodeficiency, structural issues.
- Symptoms worse past 2 years; started after nasal fracture (hockey injury) years prior.

July 7, 2026 — Allergy Testing Results

- Negative IgE testing for common allergens (White Oak, Timothy Grass, Short Ragweed, Penicillium, Mugwort, Mouse Epithelium).

July 10, 2026 — Otolaryngology Telemedicine (Lee, BWH)

- Severe ongoing symptoms including 20 lb unintentional weight loss, bright red blood in stool, thick mucus, left ear discomfort (1-2 yrs, crackling with jaw movement), facial pressure, severe hypertension requiring ER visit.
- Self-exam revealed yellowish-white mass behind drum; multiple prior ENT visits without clear etiology; ear tube did not help.

Full original reports, imaging, audiograms, operative notes, and Bebird media available from patient. This summary is for rapid ER/ENT review. Thank you for your urgent evaluation and care.